[redacted]

DOCTOR'S CASE REPORT

NAME : [redacted]

AGE / SEX : 6y / Mah

DATE : 10/3/26.

PIMS No. :

Chief Complaints:
c/o Fever x 2days.
c/o vomiting x 6days
c/o cough & Rhinorrhea x 2day.

PRESENTATION :
A 6½ y/o old mah child child com with c/o Fever (3/3/26) for 2days, c/o vomiting for for 6day, the fever was insidious in onset, intermittent in nature, aggravating on evening, relieved on medicine
A/W c/o vomiting for 6 days, 3-4 episodes per day which is non-bilious, non-blood stained, & contains food particle
c/o cough & Rhinorrhea - non-paroxysmal dry cough, more during night Time, no seasonal variation.
c/o Loose stools for 5day (5 episodes/day), non-blood stained, Large, watery.

PAST HISTORY :

PERSONAL HISTORY : NO H/O Burning Micturition, Headach, Blurring of vision

Past History:
No Previous Hospital administration.
No Previous Nebulization

FAMILY HISTORY :
Nil Significant

GENERAL EXAMINATION : Conscious, Oriented

Piccle - Pallor (P)

PR - 114/min SpO2: 98% @RA.

RR - 22/min

BP -

SYSTEMIC EXAMINATION :

CVS :

S1 & S2 Heard
No murmur

RS : BIL AE (P)

RS : NVBS (P)

ABD : Soft, non-tender
Liver Spon-11cm

ABD Rs :

CNS

NFND

CNS

ENT / EYE :

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LOCAL EXAMINATION :

Eschar present over (R) Iliac region

PROVISIONAL DIAGNOSIS :

? Enteric Fever

? Scrub Typhus.

DIFFERENTIAL DIAGNOSIS :

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INVESTIGATIONS

CBC, CRP, Blood culture, LFT, RFT

HIV, Widal, Scrub IgM.

PLAN OF CARE :

Admission

EXPECTED OUTCOMES

TREATMENT GIVEN :

IV XONE ~~100g~~ 100 mg/kg/day

INJ PAN 40g OD

IV EMESET SOS.

SIGNATURE OF RESIDENT :

NAME :

DATE :

TIME :

SIGNATURE OF CONSULTANT :

NAME :

DATE :

TIME :